

Using the system

What each person at the practice does, in the order they do it — and what the software will and will not let them do.

Applies to: Molargo practice management. One interface, two heads — the desktop app and the web app.

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Read this first: the job titles in this guide are shorthand. Molargo grants access by *permission* and by *ownership*, not by job title. Section 2 sets out exactly what that means, and it is the part most likely to surprise you.

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1 Before anyone signs in

What Molargo is

One patient record, with the diary, the chart, billing, prescribing and the worklists hanging off it. The database is a file on the machine, so the surgery keeps working when the connection does not.

What it is not

Stated plainly, because a manual that describes features the software lacks sends people hunting for buttons that were never there.

NOT BUILT	WHAT THAT MEANS ON THE DAY
Online booking, patient portal	Every appointment is made by a person, in the diary.
Payment gateway	No card is ever taken. Every payment recorded is money that arrived somewhere else.
Health fund claiming	Claims are recorded and tracked. Submitting them needs HICAPS or a fund's own channel.
Electronic prescriptions	No eRx token. The printed, signed sheet is the script.
Syncing between machines	Each installation is its own database. Nothing merges.

NOT BUILT

WHAT THAT MEANS ON THE DAY

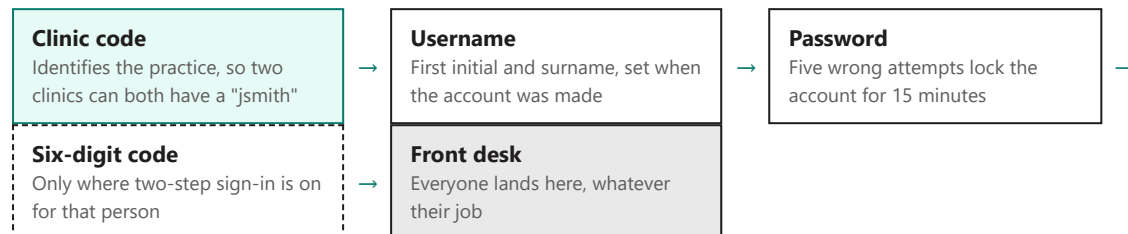
Campaign sending

Templates and audiences exist; the send does not.

Anything on a timer

Reminders go when somebody presses the button. Nothing runs unattended.

Signing in



BEFORE YOU GO LIVE

Password resets and two-step codes are emailed through the practice's own mail account, set up at [Admin → Settings](#). Until that exists nobody can reset their own password — somebody with staff access has to set a new one on their record. Two-step sign-in cannot be switched on at all without it.

Every sign-in attempt, successful or not, is written to [Admin → Audit log](#).

2 How access really works

Most practice software gates screens by job title: a receptionist sees the receptionist's menu. Molargo does not work that way, and knowing this before you set up your staff will save you an argument later.

Three separate things decide what somebody can do.

THE THING	WHAT IT DECIDES	WHAT IT DOES NOT DECIDE
Clinic tenant	Which practice's records exist for you at all. Enforced in the database, not the screen.	Nothing within your own practice.
Role Dentist, Hygienist, Administration...	Whether you can be booked into a chair, whether you count as a billable clinician seat, and — in the places that check — whether you are answerable for a clinical record.	Which menus you see. No role hides a section, with one exception (3.7).
Permissions five flags	Every administrative write in the app: staff, pricing, sites, stock costs, settings.	Clinical work. A dentist with no permissions still charts, prescribes and bills.
Owner one person	Everything, implicitly — plus the two things no permission can grant.	—

THE PRACTICAL CONSEQUENCE

A receptionist account can open Charting and look at a tooth chart. Nothing in the software stops them. What stops them is that they are not bookable, they hold no administrative permission, and a certificate will refuse to issue in their name because they have no licence number on file. **Molargo's confidentiality boundary is the clinic, not the job title** — and every record opened is written to the audit log, which is how "who looked at this" gets answered.

If you need a screen-level boundary between front desk and surgery, it does not exist yet. Say so to your team rather than assuming it.

2.1 Roles, and what each one changes

ROLE	ANSWERABLE FOR CLINICAL RECORDS	BOOKABLE IN THE DIARY	COUNTS AS A PAID CLINICIAN SEAT
Dentist	Yes	Yes	Yes
Hygienist	Yes	Yes	Yes
Oral health therapist	Yes	Yes	Yes
Dental therapist	Yes	Yes	Yes
Prosthetist	Yes	Yes	Yes
Specialist (<i>endodontist, periodontist, orthodontist</i>)	Yes	Yes	Yes

ROLE	ANSWERABLE FOR CLINICAL RECORDS	BOOKABLE IN THE DIARY	COUNTS AS A PAID CLINICIAN SEAT
Dental assistant	No	No	No — free
Administration (<i>front desk, practice management</i>)	No	No	No — free
Super admin (<i>the vendor, not a clinic</i>)	No	No	Not a clinic account

An assistant is excluded from clinical authorship despite working chairside: they assist with treatment but do not diagnose, prescribe or certify, and the record has to name whoever is answerable for the decision.

Assistants and reception being free is not generosity. Charging per front-desk login is what makes a practice share one reception account, and a shared account makes the audit log worthless.

2.2 The five permissions

Granted per person at `Admin → Users`. A new account gets none of them — clinical work only, nothing administrative.

PERMISSION	UNLOCKS	WORTH KNOWING
Manage staff	Admin → Users and Roles: adding people, editing them, setting passwords	The most powerful of the five by some distance. Anyone who can set another person's password can sign in as them. Grant it last.
Manage pricing	The item catalogue's editor: fees, per-site prices, adding and withdrawing items	Editing only. <i>Reading</i> the catalogue is open to everyone — the front desk quotes from it all day.
Manage sites	Admin → Sites: locations, their details, opening hours, and the diary's chairs	Opening hours set here are what the diary refuses bookings against.
Manage inventory	Stock cost prices, suppliers, purchase orders	Not receiving stock or marking it used — that is surgery and front-desk work and stays open to all. This gates the money side.
Manage settings	Admin → Settings: the mail account, notification rules, printers	Hands over the ability to send email as the practice. The app password is stored in plain text in the database file.

Holding any one of *manage staff*, *manage sites* or *manage settings* puts Admin in the top bar. Pricing and inventory do not: those are edited on Billing and Inventory, outside the Admin section, so holding them is no reason to show an Admin tab whose every pane would refuse.

2.3 What only the owner can do

OWNER-ONLY	WHY IT IS NOT A GRANTABLE PERMISSION
Change who owns the practice, and who holds which permissions	If it were grantable, whoever held it could grant themselves every other permission and then ownership — which would make the rest of this section decoration.
Export the whole database	The file is a complete, unencrypted copy of every patient record the practice holds. Whoever can produce one can walk out with the practice.

THERE IS NO WAY BACK IN

There is no vendor override and no master password. The owner's permissions are computed, not stored, precisely so an owner cannot be left holding a partial set and locked out of their own practice. Keep the owner account recoverable: a working mail account, and a second person with *manage staff* who could set a password if the owner is hit by a bus.

2.4 Suggested grants

THE JOB	ROLE	PERMISSIONS
Practice owner	Dentist (or whatever they practise as)	None needed — ownership carries everything
Associate dentist	Dentist	none
Hygienist	Hygienist	none
Practice manager	Administration	staff sites settings pricing inventory
Senior receptionist	Administration	inventory — so they can order
Receptionist	Administration	none
Dental assistant	Dental assistant	none

Two rules the software enforces whatever you choose: email and mobile must each be unique within the practice — a number that reaches two people is a code going to whichever of them a query found first — and the last active clinician cannot be deactivated.

3 The day, by role

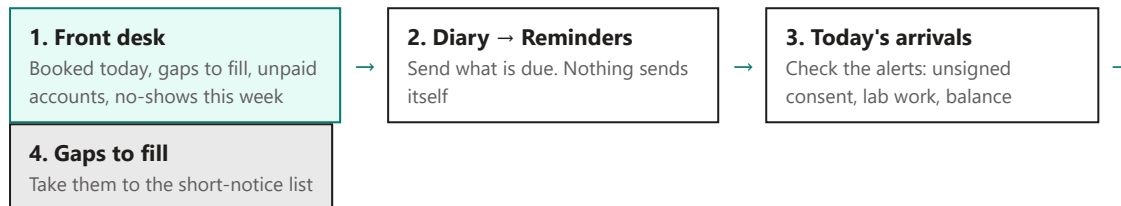
Everyone lands on the front desk. What differs is where they go next, and which of the day's records they are the one to create.

3.1 Receptionist / front desk

Role: Administration · Permissions: usually none · Free seat

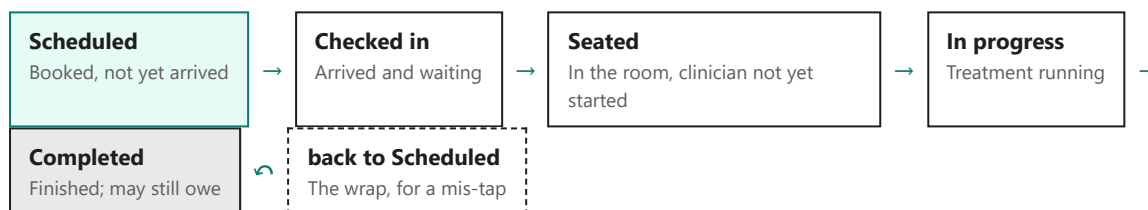
The busiest account in the building, and the one that touches every transaction before and after the clinician does.

The morning



As each patient arrives

Tap the status chip on the arrivals list to advance it. The ladder wraps, so an accidental tap can be walked round rather than undone.



Cancelled and *Failed to attend* sit outside the ladder and a stray tap cannot walk them back into it. A no-show is tracked separately from a cancellation because it costs the practice the slot and feeds the patient's missed-appointment count.

Taking a booking on the phone

1. Search the patient from the top bar, or **Patients → New patient** if they are new.
2. **Diary → New appointment**. Patient, type, provider, chair, time.
3. Read the pre-booking panel on the right before you save: medical alerts, outstanding lab work, account balance, missed-appointment history.
4. Tick **Tell the patient** — email, text, or both. Each box explains itself when it cannot be used.
5. Save. What went out is written to the patient's communication log, including anything that failed.

The form warns rather than blocks for a double-booking — sometimes that is deliberate — but it refuses a day the practice is closed.

Taking money

Billing → Invoices, raise from the fee catalogue, record the payment against it. No card is taken anywhere in this app: you are recording money that arrived by terminal, transfer or cash. **Billing → End of day** is the reconciliation, **Debtors** the chase list.

What a receptionist cannot do

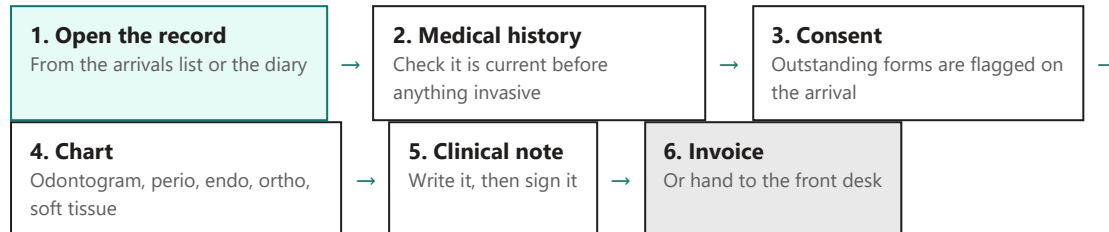
not bookable They cannot appear as the provider on an appointment. **no certificate** A certificate will not issue in their name — it needs a licence number on the record. **no admin** With no permissions, Admin is absent from the top bar.

3.2 Dentist

Role: Dentist · Permissions: usually none · Billable clinician seat

A dentist with no permissions at all can do every piece of clinical and billing work below. Permissions are about administering the practice, not treating patients.

The chairside loop, one patient



WHAT SIGNING A NOTE MEANS HERE

A signed clinical note *is* the record. Later changes are additions, not edits — the original text stays. A note that could be quietly rewritten is a note nobody can rely on in a complaint, which is the only moment it really matters.

The rest of a dentist's screens

SCREEN	WHAT THEY DO THERE
Charting	The worklist of today's chairside work, and the chart itself per patient.
Rx & referrals	Prescriptions, outbound and inbound referrals, medical certificates, the formulary.
Treatment plans	Built from the chart, presented chairside, accepted with a signature, then booked as visits.
Diary	Their own column in the day view; booking follow-ups before the patient leaves the chair.
Billing	Raising the invoice for what was just done.
Reports	Production, procedure mix, new patients.

Before issuing a certificate

The clinician must have a **licence number** on their staff record. Without one the issue is refused — a certificate signed by somebody with no licence number is not a document an employer can rely on, and issuing one puts the practice's name to a claim it cannot support. Set it at [Admin → Users](#), which usually means asking the owner or practice manager.

3.3 Hygienist, oral health therapist, dental therapist

Role: as registered · Permissions: usually none · Billable clinician seat

The same flow as 3.2, with the same authority to chart, note, prescribe and certify within their own registration. Molargo does not police scope of practice for you — it records who was answerable, and your registration decides what you may do. Two practical differences:

- **Recalls are their engine.** Scale & polish carries a six-month recall interval, so completing a hygiene visit is what puts the patient back on the worklist. See 4.1.
- **The patient's own interval wins.** A periodontal patient on three months does not revert to six because they came in for a routine clean.

3.4 Dental assistant

Role: Dental assistant · Permissions: usually none · Free seat

Chairside support. Not bookable, not answerable for clinical records, not a paid seat.

- **Inventory → Stock** — use and adjust, so what came off the shelf is recorded as it happens.
- **Inventory → Sterilisation** — running and releasing loads, and tying a load to the patients it was used on.
- **Inventory → Lab** — cases out and back.
- Preparing the record: opening the chart, checking consent is signed, seeing the day's list.

Recording stock use and receiving deliveries is deliberately open to everyone. What needs the *manage inventory* permission is the money side — cost prices, suppliers and purchase orders.

3.5 Practice manager

Role: Administration · Permissions: staff, sites, settings, pricing, inventory · Free seat

Everything in 3.1, plus the Admin section. This is the account that sets the practice up and keeps it running.

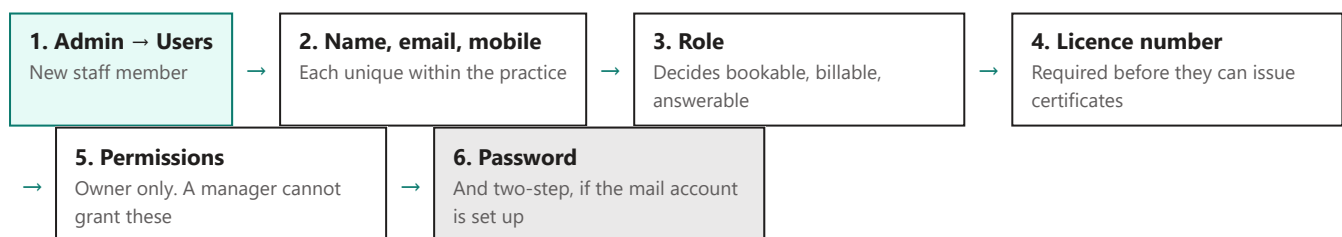
Setting up a new practice

After signup, **Setup** appears in the top bar with the number of essential steps outstanding, and disappears once they are done. Each step says what is missing, what happens while it is, and links to the screen that owns it.

STEP	WHERE	WHY IT MATTERS
Address and phone	Admin → Sites	They head every receipt, referral and script the practice prints.
Opening hours	Admin → Sites	The diary is drawn against them and refuses bookings outside them. Until set, the app assumes Mon–Sat 08:00–18:00.
Chairs and rooms	Admin → Sites	One column of the day view per chair. Name them as the staff say them.
The team	Admin → Users	Every note, script and invoice is signed by whoever is signed in. A shared login makes the audit trail worthless.
Mail account	Admin → Settings	Password resets, two-step codes, receipts and reminders all go through it.
Reminders	Diary → Reminders	The cheapest thing in the app — a reminder the day before turns no-shows into arrivals or refillable cancellations.
Text messaging	Admin → Plan	Optional and charged per message. A cost decision, which is why it sits with the plan.
First patient, first appointment	Patients, Diary	Nothing was invented for you. A new practice starts with no patients and no bookings.

A new practice does arrive with the medical history questionnaire, consent forms, message templates, stock categories, the fee catalogue, the appointment types and the dental formulary already filled in. All editable; none of it on the checklist.

Adding a member of staff



THE SPLIT AT STEP 5

A practice manager with *manage staff* can add people, edit them and set passwords. They cannot change who holds which permissions, and cannot make anyone the owner — that is checked against the stored record rather than what is being saved, so it cannot be smuggled through an edit. Adding a colleague is not a route to granting yourself the practice.

The rest of the month

- **Admin → Plan** — what the subscription costs, seats and sites counted, and the switch for text messaging.
- **Admin → Audit log** — searchable, paged, and it records views as well as changes.
- **Billing → Item catalogue** — the fees, with per-site prices where they differ.
- **Inventory → Orders** — purchase orders, recorded rather than sent.

- **Admin → Data** — the local database, its size and its backups.

3.6 Practice owner

Role: whatever they practise as · Permissions: all, implicitly · Billable if clinical

Everything above, plus the two things no permission grants: changing who owns the practice and who administers what, and exporting the whole database.

The owner is created by signup and cannot be left half-privileged. Their permissions are computed rather than stored, so a stray click — or a permission added to the software after their account was made — cannot lock them out of their own practice. There is nobody who could let them back in.

Owner's monthly ten minutes

1. **Reports → Overview** — production, and where it came from.
2. **Billing → Debtors** — what is owed and how old it is.
3. **Admin → Plan** — seats counted from active clinical staff, so a departure that was never deactivated is money.
4. **Admin → Audit log** — who has been looking at what.
5. **Admin → Data** — take a backup off the machine. It is the only copy.

3.7 Vendor operator (super admin)

Role: Super admin · Belongs to the vendor, not to any clinic

This is the only role in Molargo that actually restricts anything. A super admin sees a different top bar entirely — Clinics, Plans, Billing, SMS, Email, Users, Help — and none of the clinical sections, because the vendor's own tenant has no patients, no diary and no invoices.

A practice cannot grant this role: its own Users screen does not offer it, and accounts holding it sit outside the practice's records altogether.

VENDOR SCREEN	WHAT IT IS FOR
Platform → Clinics	Every subscribing practice, its plan and its state.
Platform → Plans	The plans on sale, per country, with prices and included text allowances.
Platform → Billing	Raising monthly subscription charges, including the previous month's text usage.
Platform → SMS	The gateway per country, its rate, its credit limit, and usage across all subscribers.
Platform → Email	The vendor's own sending account — the one thing that can reach somebody before their clinic exists, which is what a signup code has to do.
Platform → Help	The knowledge base every practice reads. Edited here, so a correction needs no release.

4 Transactions end to end

The flows below cross more than one pair of hands. Each lane is who does that step; the numbers run in order down the page.

4.1 New patient → first visit → invoice → recall

The spine of the system. Everything else hangs off it.

FRONT DESK	1 Create the patient. Patients → New patient. The form flags likely duplicates as you type — same name, same date of birth, same number — because a second record for one person splits their history in half. 2 Book the appointment. Type, provider, chair, time. Read the pre-booking panel. Tick Tell the patient.
SYSTEM	3 Writes the confirmation to the patient's communication log — including a message that failed, because that log is the answer to "were they told".
FRONT DESK	4 The day before: Diary → Reminders → Send what is due. Running it twice is harmless — an attempt already made is never repeated, so an interrupted run can simply be run again. 5 On arrival: advance the status to Checked in, then Seated.
CLINICIAN	6 Medical history — taken or confirmed. The questionnaire ships with the practice. 7 Treat and chart. Odontogram or perio, per tooth and per surface. 8 Write and sign the note. After signing, changes are additions. 9 Advance the appointment to Completed.
SYSTEM	10 Completing a visit creates or rolls forward a recall, if the appointment type carries an interval. Exam & clean and Scale & polish do; a crown fit, an extraction and a root canal do not — those finish a course of treatment rather than starting the next one.
FRONT DESK	11 Raise the invoice from the fee catalogue and record the payment. The ledger is the truth; the total on the invoice follows it. 12 Book the next visit while they are standing there. Booking at or near the recall's due date marks it Booked and takes it off the worklist.

THREE RULES WORTH KNOWING ABOUT RECALLS

One per patient. Finishing another visit rolls the existing recall forward rather than adding a second — two rows would be two people ringing the same patient in the same week.

An early booking does not clear it. Somebody coming in next week for a broken tooth has not had their check-up.

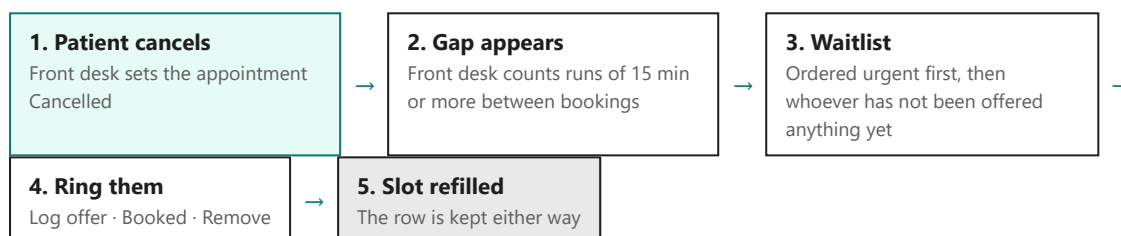
A patient who has declined recalls is never put back on the list by attending.

4.2 Treatment plan → presentation → consent → treatment

CLINICIAN	1 Start from the chart. The findings already recorded become the plan's starting items, rather than being retyped. 2 Build it into visits — what happens on the first appointment, what on the second. Fees come from the catalogue.
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	3 Present it chairside. The presentation screen shows the plan and its alternatives as the patient sees them.
PATIENT	4 Accepts, or declines. Acceptance records who agreed and in what capacity — Self, Parent, Guardian — plus a signature where a tablet captured one. Consent for a child is given by somebody else, and a record that does not say who is not evidence.
FRONT DESK	5 Book the visits the accepted plan calls for. Each booking is linked back to the plan visit it covers.
SYSTEM	6 Booking a plan visit raises the consent forms its procedures need. Anything still unsigned is flagged on the arrivals list, so the front desk sees it before the patient sits down.
CLINICIAN	7 Consent signed, then treat. A visit whose procedures need no separate consent says so, rather than showing an empty list that looks like a loading failure.

4.3 A cancellation, and filling the gap

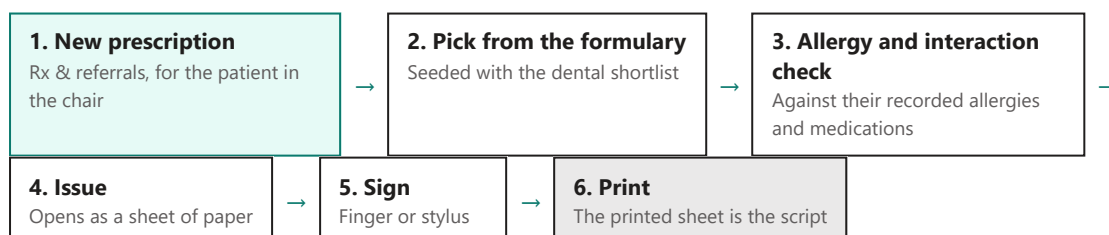


Getting somebody onto the short-notice list takes seconds: **Diary → Waitlist → Add someone**. Everything except the patient is optional, because whoever is taking a cancellation call has thirty seconds.

Three buttons per row, because an offered slot has three outcomes. **Log offer** records that you rang and leaves them on the list. **Booked** and **Remove** take them off — and the row is kept either way, so "you never call me" has an answer.

Offers are made by hand. Automatic offers and first-reply-wins would need a messaging gateway racing replies, which is not built.

4.4 Prescription



WHAT THE CHECK IS, AND IS NOT

The interaction screen is written by hand against the medicines in the formulary — it is not a pharmacological database, and the prescribing screen says so on the screen. Treat it as a prompt, not an authority.

Nothing is transmitted. An eRx token needs a prescription-exchange gateway, which is not built.

4.5 Medical certificate

CLINICIAN	1 Draft it. Rx & referrals → Certificates. Set the days unfit, and — if it is not today — the day they are unfit from. A patient seen late on a Friday is often unfit from Monday. Five days is the cap; beyond that they need their own doctor.
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	2 Edit the wording while it is a draft — "light duties only", "unfit for heavy lifting". Keep the treatment out of it: an employer has no right to the clinical detail, which is why the standard wording says "dental treatment" and stops. 3 Issue. Refused if the clinician has no licence number on file. 4 Sign and print. A certificate printed before it is issued carries a visible "Draft — not issued" band.
SYSTEM	5 Once issued, the wording is fixed. Rewriting a document the patient already has would change what the practice said after it said it. Draft a new one instead. 6 An issued certificate nobody signed is flagged in the list — an unsigned sheet is one an employer has no reason to accept.

Deleting a certificate takes it off the list, not out of the record: the wording is kept, and the audit log names who removed it and what it covered.

4.6 Month end: texts, usage and the subscription charge

PRACTICE	1 Texts go out through the month — reminders, recalls, booking confirmations. Admin → Plan shows the rate, how many have gone this month and what they come to.
SYSTEM	2 Counts only messages that actually went. Queued, failed and suppressed ones are not charged for.
VENDOR	3 Platform → Billing raises the month's charges: the plan, plus sites beyond the base, plus clinician seats beyond the allowance. 4 Adds last month's text usage above the included allowance — a month is billed once it is finished and the total cannot move.
NOBODY	5 Nothing collects money. There is no invoice, no payment gateway and no renewal date. Every Paid or Failed on a charge was recorded by a person after money did or did not arrive somewhere else.

5 Where the system stops

Every line here is a deliberate absence, not a defect. Knowing them is what stops somebody waiting for a message the software was never going to send.

AREA	WHAT WORKS	WHAT DOES NOT
Reminders and recalls	Cadence, due list, per-appointment record of every attempt	Nothing runs on a timer. Somebody presses <i>Send what is due</i> , or nothing goes.
Short-notice list	Priority ordering, offer logging, three outcomes	No automatic offers, no first-reply-wins.
Billing	Invoices, payments, debtors, end of day	No card is taken. No payment gateway exists.
Claims	Recording and tracking	No submission. That needs HICAPS or a fund's channel.
Prescribing	Formulary, hand-written interaction checks, signed printed script	No eRx token, no pharmacological database.
Inventory	Stock, movements, reorder flags, purchase orders	Nothing is ordered. Purchase orders are recorded, not sent.
Comms	Templates with merge fields, the automated set, the log	Campaign sending is not built.
Access control	Clinic isolation, five permissions, ownership, full audit	No screen-level separation by job title. A receptionist can open a chart; the audit log is what records that they did.
Data	One database file, backups, owner-only export	No syncing between machines. The database is not encrypted. Losing the machine loses the practice unless a backup left it.
Credentials	Hashed passwords, lockout, optional two-step by email	The mail app password is stored in plain text. Treat the database file as carrying it.

6 Reference

6.1 Screen map

Sections in top-bar order. Tabs can be linked to directly — `/admin?tab=sites` , `/diary?tab=reminders` — which is useful in a practice's own written procedures.

SECTION	ADDRESS	TABS
Setup	<code>/setup</code>	Appears only while essential steps are outstanding
Front desk	<code>/</code>	Arrivals, tasks, short-notice list, surgery messages
Diary	<code>/diary</code>	Diary (day / week / month / list), Recalls, Waitlist, Reminders
Patients	<code>/patients</code>	Overview, Medical history, Documents, Comms log, Billing, Privacy
Charting	<code>/charting</code>	Odontogram, Perio, Endo, Ortho, Soft tissue, Notes timeline
Rx & referrals	<code>/rx</code>	New prescription, Rx history, Outbound, Inbound, Certificates, Formulary

SECTION	ADDRESS	TABS
Billing	<code>/billing</code>	Invoices, Item catalogue, Claims, Debtors, Plans, End of day
Inventory	<code>/inventory</code>	Stock, Purchase orders, Lab cases, Sterilisation, Equipment, Suppliers, Categories
Comms	<code>/comms</code>	Templates, Campaigns, Automated, Reviews, Inbox, Preferences
Reports	<code>/reports</code>	Overview, New patients, Operations, Procedure mix, Builder
Admin	<code>/admin</code>	Users, Roles, Sites, Registrations, Rosters & pay, Plan, Audit log, Security, Data, Medical history, Consent forms, Appointment types, Settings
Help	<code>/help</code>	The knowledge base, by category

6.2 Appointment statuses

STATUS	MEANS
Scheduled	Booked, not yet arrived.
Confirmed	Reminder sent and the patient replied to confirm.
Checked in	Arrived and waiting.
Seated	In the room, clinician not yet started. The cue that the room is occupied but the appointment is not yet running late.
In progress	In the chair, treatment running.
Completed	Treatment finished. May still owe payment. This is what creates the recall.
Cancelled	Cancelled with notice, so the slot could be offered on.
Failed to attend	Did not arrive and did not cancel. Feeds the patient's missed-appointment count.

6.3 Glossary

TERM	IN THIS SYSTEM
Clinic code	The practice's short name, typed at sign-in. Derived from the practice name at signup, not chosen.
Owner	The one account that holds every permission implicitly and can change who holds what.
Permission	One of five administrative flags. Nothing to do with clinical work.
Role	What somebody is registered to do. Decides bookable, billable and answerable.
Recall	One per patient. Created when a visit with a recall interval completes; cleared by booking near the due date.
Short-notice list	The waitlist. Who to ring when a slot frees up.
Cadence	Days before an appointment at which reminders are due — "7, 1" means a week out and again the day before.
Soft delete	What every deletion in this app is. The row stays, marked deleted, and the audit entry names who removed it and what it was.
Seat	An active staff member in a clinical role. What the subscription counts.

WHERE THIS GUIDE LIVES

The same material, kept current and searchable, is in the app at [Help](#). It is edited by the vendor and reaches every practice at once, so where this printed copy and the Help screen disagree, the screen is newer.